Advanced ALK-positive NSCLC after crizotinib Adults with ALK-positive advanced non-small-cell lung cancer previously treated with crizotinib: NCCP 00340a. This suite does not turn the broader first-line licence into an NCCP indication or funding approval. Confirm your treatment setting with the team.

NCCP 00340 · VERSION 4

Ceritinib monotherapy

The usual starting dose is 450 mg once daily with food, every day without a planned rest week. Follow your own prescription. Treatment continues while it helps and side effects remain acceptable; there is no fixed course length.

BASELINE → TREATMENT → FOLLOW-UP

Ceritinib checks over time

Columns are milestones, not equal durations. The team books actual tests and reviews; no patient-entered dates are needed.

Check	Before starting	First 3 months	After month 3	Benefit review	Hold / change / stop
Daily ceritinib	Confirm ALK, previous crizotinib and prescription	Daily with food; no planned rest week	Continue authorised daily schedule	Assess benefit and tolerability	Written instructions for any change
Liver tests	ALT, AST and bilirubin	Every 2 weeks	Monthly; sooner if abnormal	Continue tests and symptom review	Check recovery before any restart
Blood count, kidneys and glucose	FBC, renal profile and fasting glucose	Every 4 weeks; glucose sooner if needed	Every 4 weeks and when indicated	Review diabetes, symptoms and medicines	Follow unresolved abnormalities
Heart and pancreatic checks	ECG, pulse/BP, lipase and amylase	ECG after 1 month; other checks as needed	ECG/electrolytes and pancreatic enzymes as indicated	Review pulse/BP and symptoms	Investigate fainting or abdominal pain
Symptoms, interactions and supply	Review medicines, food and contact plan	Report breathing/GI symptoms promptly	Check adherence, prescriptions and supply	Scans and disease assessment per consultant	Confirm follow-up and who authorises restart

THE WHOLE TREATMENT JOURNEY

Ceritinib monotherapy: treatment and checks

ALK-positive advanced NSCLC · after crizotinib

The usual starting dose is 450 mg once daily with food, every day without a planned rest week. Follow your own prescription. Treatment continues while it helps and side effects remain acceptable; there is no fixed course length.

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before starting	First 3 months	After month 3	Benefit review	Hold / change / stop
Daily ceritinib NCCP 00340 · SMPC	Confirm ALK, previous crizotinib and prescription	Daily with food; no planned rest week	Continue authorised daily schedule	Assess benefit and tolerability	Written instructions for any change
Liver tests NCCP 00340 · SMPC	ALT, AST and bilirubin	Every 2 weeks	Monthly; sooner if abnormal	Continue tests and symptom review	Check recovery before any restart
Blood count, kidneys and glucose NCCP 00340 · SMPC	FBC, renal profile and fasting glucose	Every 4 weeks; glucose sooner if needed	Every 4 weeks and when indicated	Review diabetes, symptoms and medicines	Follow unresolved abnormalities
Heart and pancreatic checks NCCP 00340 · SMPC	ECG, pulse/BP, lipase and amylase	ECG after 1 month; other checks as needed	ECG/electrolytes and pancreatic enzymes as indicated	Review pulse/BP and symptoms	Investigate fainting or abdominal pain
Symptoms, interactions and supply NCCP 00340 · SMPC	Review medicines, food and contact plan	Report breathing/GI symptoms promptly	Check adherence, prescriptions and supply	Scans and disease assessment per consultant	Confirm follow-up and who authorises restart

What each check is for

1 Safe start

Before first dose

Correct biomarker, treatment line and safety context matter.

Ask / act: Confirm dose, food instructions, support medicines and contacts.

Sources: NCCP 00340 · SMPC

2 Early liver surveillance

Every 2 weeks for 3 months

Liver injury may occur without early symptoms.

Ask / act: Confirm test bookings and result reviewer.

Sources: NCCP 00340 · SMPC

3 Regular clinical checks

Throughout daily treatment

Continuous tablets still need continuing review.

Ask / act: Review symptoms, interactions, intake, supply and scans.

Sources: NCCP 00340 · SMPC

4 Dose held or treatment stopped

Toxicity, progression or changed priorities

Restart criteria differ by toxicity and food schedule.

Ask / act: Prescriber gives a new written plan and follow-up.

Sources: NCCP 00340 · SMPC

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

SACTCheck · Developed by Dr Paul O'Brien · v0.84.25 · Sources checked 2026-10-29
Source reconciled; independent clinical, pharmacy and patient/PPI review pending